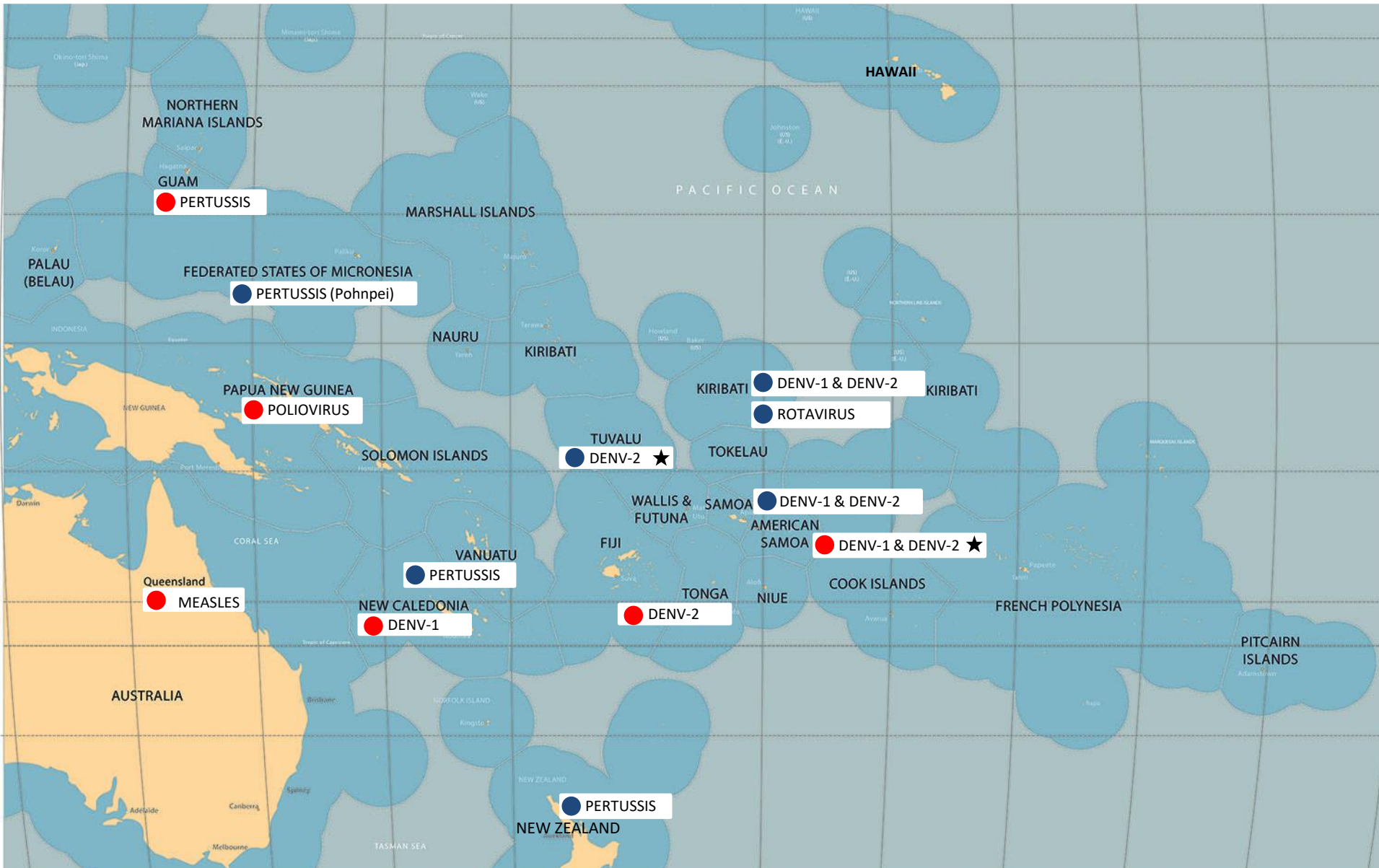


Epidemic and emerging disease alerts in the Pacific as of 26 May 2026



- Legend**
- Red dot: Cases reported are increasing or peaking
 - Blue dot: Cases reported are decreasing or circulation is ongoing.
 - Grey dot: Awaiting confirmation of aetiology.
 - Star: No update for more than 2 weeks

DENV – Dengue Virus

An interactive version of the map is available online at:
www.spc.int/phd/epidemics/



Pacific Community
 Communauté du Pacifique
 For the Pacific Public Health Surveillance Network (PPHSN)

Highlights/updates since the last map was sent on PacNet on 19 May 2026:

Dengue

- **Cook Islands:** Dengue transmission in the Cook Islands has been interrupted, with no new cases reported since 28 April 2026. The Ministry of Health officially declared the country dengue-free on 25 May 2026 after more than two incubation periods (26 days) without any reported cases, meeting the criteria for declaring the end of the dengue outbreak. This follows the scale-down of Operation Namu26 on 24 April 2026, with routine surveillance and vector control measures continuing to sustain the dengue-free status. **The blue alert for DENV-1 and DENV-2 is removed.** – Source: [Cook Islands Dengue Free - Te Marae Ora Cook Islands Ministry of Health](#) accessed on 26 May 2026.
- **Kiribati:** Dengue transmission continues in Kiribati, with the outbreak ongoing since April 2025. As of 26 May 2026, a cumulative total of 535 laboratory-confirmed cases and 1,583 suspected cases have been reported nationwide, including 20 new confirmed cases and 19 new suspected cases since 13 April 2026. Transmission remains concentrated in South Tarawa, with both DENV-1 and DENV-2 circulating, and most cases reported as mild, although one death and 68 hospitalisations have been recorded to date. **The blue alert for DENV-1 and DENV-2 remains in effect.** – Source: *Dengue Fever Situational Report 33 shared with PPHSN Coordinating Body Focal point on 26 May 2026.*
- **New Caledonia:** As of 23 May 2026, dengue transmission remains ongoing in New Caledonia, with 1,837 cases reported in 2026. The outbreak is driven by locally acquired infections, with DENV-1 as the predominant serotype. Environmental conditions favourable to mosquito breeding, together with population movement, continue to support transmission, and health authorities are reinforcing early care-seeking and vector control measures. **The red alert for DENV-1 remains in effect.** – Source: [La dengue | Direction des Affaires Sanitaires et Sociales de Nouvelle-Calédonie](#) accessed on 26 May 2026.
- **Samoa:** As of EpiWeek 20 (11–17 May 2026), dengue transmission continues in Samoa, with 141 new cases reported, representing a 7% increase compared with the previous week, and eight new hospital admissions with no ICU cases or deaths. DENV-1 (54%) and DENV-2 (46%) continue to co-circulate, with transmission widespread across Upolu (64%) and Savai'i (36%), and children under 15 years accounting for the majority of cases (74%). Since 1st January 2025, a cumulative total of 19,029 clinically diagnosed cases and 5,446 laboratory-confirmed cases have been reported, with nine dengue-related deaths to date, while public health measures continue to focus on community awareness, vector control and early healthcare seeking. **The blue alert for DENV-1 and DENV-2 remains in effect.** – Source: [Dengue-sitrep-issue-no-57](#) accessed on 26 May 2026.
- **Tonga:** As of 25 May 2026, dengue transmission continues in Tonga, with a total of 66 laboratory-confirmed cases reported, including two new cases since the 22 May 2026. Two cases are currently hospitalised, with no dengue-related deaths reported to date. The epidemiological profile remains unchanged, with the highest burden among children aged 11–14 years, predominantly male, and most cases reported from Tongatapu. DENV-2 remains the identified circulating serotype, and response activities, including community awareness campaigns and vector control measures, are ongoing to limit further transmission. **The red alert for DENV-2 remains in effect.** – Source: [Dengue Fever Outbreak Situation Report #23 - Ministry of Health Tonga - Official Facebook page](#) accessed on 26 May 2026.

Pertussis/Whooping cough

- **New Zealand:** Pertussis activity in New Zealand continues as part of the ongoing national epidemic, with transmission persisting nationwide. As of the week ending 15 May 2026, 48 cases were reported, an increase compared with 44 cases in the previous week. This brings the cumulative total for 2026 to 734 cases. **The blue alert for pertussis remains in effect.** – Source: [Pertussis dashboara](#) accessed on 26 May 2026.

Other Information:

Dengue

- **Federated States of Micronesia, Kosrae State:** Dengue activity in Kosrae State, Federated States of Micronesia, remains under monitoring, with three cases reported over the past two weeks, including two newly confirmed and unlinked cases detected through laboratory surveillance. The affected individuals have been managed clinically, with cases reported as isolated, treated and discharged, and no evidence of widespread transmission identified to date. Given the presence of the mosquito vector on the island, public health authorities are maintaining surveillance and implementing vector control and community clean-up measures to reduce the risk of further transmission. – Source: *FSM weekly surveillance report (May 4-14, 2026 – EpiWeek 19 & 20) shared with PPHSN Coordinating Body Focal point on 22 May 2026.*
- **New Zealand:** In New Zealand, during EpiWeek 20 (16 – 22 May 2026), two confirmed imported dengue cases were reported. The confirmed cases had each been to the Cook Islands and Samoa during the incubation periods. Given the known serological cross-reactivity between dengue and other flaviviruses, some reported dengue cases may ultimately be reclassified as other flavivirus infections as epidemiological and laboratory investigations continue. – Source: *NZ Arbovirus Notifications by Country (20: 16/05/2026–22/05/2026) shared with PPHSN Coordinating Body Focal point on 25 May 2026.*
- **Vanuatu:** From EpiWeek 01 to EpiWeek 19, 2026, a total of eight dengue cases have been reported in Vanuatu, all on Efate Island, with no hospitalisations or deaths recorded. Cases are primarily concentrated in the Pango Area Council (88%), with the remaining cases reported in Port Vila, and epidemiological investigations have identified links between several cases, including recent travel to Fiji within the incubation period. Although case numbers remain below the alert threshold and no outbreak has been declared, a slight increase in cases has been observed and health authorities are continuing close surveillance to monitor potential changes in transmission. – Source: *Vanuatu Dengue Situation Update 01 (18 May 2026) on PacNet on 26 May 2026.*

Diphtheria

- **Australia:** Diphtheria activity in Australia has increased markedly since late 2025, with 221 confirmed cases reported in 2026 as of 18 May, representing a substantial rise compared with previous years. The outbreak is concentrated in the Northern Territory (60.2%) and Western Australia (35.7%), with most cases locally acquired and occurring in remote and very remote areas, particularly among Aboriginal and Torres Strait Islander populations.

Both cutaneous (67.9%) and respiratory (31.7%) presentations are reported, with no confirmed diphtheria-related deaths reported to date in 2026, although one death remains under investigation, and public health response measures including enhanced surveillance, vaccination, contact tracing and infection control are ongoing. – Sources: [Diphtheria outbreak update](#), [Diphtheria declared a Communicable Disease Incident of National Significance](#) and [Diphtheria in Australia – Epidemiological update – 18 May 2026](#) accessed on 26 May 2026.

Ebola virus

- **Democratic Republic of Congo and Uganda:** As of 21–22 May 2026, the outbreak of Ebola disease caused by Bundibugyo virus in the Democratic Republic of the Congo (DRC) and Uganda remains ongoing and has been designated a Public Health Emergency of International Concern by WHO on May 16, 2026, with 85 confirmed cases and 10 confirmed deaths reported across both countries, including two imported confirmed cases and one death in Uganda. In DRC, 83 confirmed cases and 746 suspected cases, including 176 suspected deaths, have been reported across Ituri, North Kivu and South Kivu provinces, with transmission concentrated in Mongbwalu, Rwampara and Bunia. High population mobility with humanitarian challenges, weak contact follow-up and inadequate isolation and referral systems continue to complicate response efforts. With no licensed vaccines or specific therapeutics currently available for Bundibugyo virus disease, response measures continue to focus on surveillance, contact tracing, infection prevention and control, case management, safe and dignified burials, and cross-border coordination, while WHO temporary recommendations do not currently advise travel or border closures. – Sources: [Epidemic of Ebola Disease caused by Bundibugyo virus in the Democratic Republic of the Congo and Uganda determined a public health emergency of international concern](#), [Ebola disease caused by Bundibugyo virus, Democratic Republic of the Congo & Uganda](#) and [Ebola disease caused by Bundibugyo virus – Democratic Republic of the Congo](#) accessed on 26 May 2026.

Murray Valley Encephalitis

- **Australia, Northern Territory:** Recent detections of Murray Valley encephalitis (MVE) virus have been reported in the Northern Territory, including two confirmed cases in Alice Springs, both of whom died, and virus detection in mosquitoes in the Greater Darwin region. Increased rainfall during the wet season has contributed to higher mosquito populations, elevating the risk of transmission of this rare but potentially severe mosquito-borne infection affecting the central nervous system. Enhanced mosquito surveillance and control activities are ongoing, and clinicians are advised to consider MVE in patients presenting with compatible symptoms, particularly during the seasonal risk period from February to July. – Source: [Murray Valley encephalitis | NT Health](#) accessed on 26 May 2026.